WASH KNOWLEDGE, ATTITUDES & PRACTICES (KAP) ASSESSMENT

Pre- and Post-Intervention Comparative Report

Host Community Households — Governorates X, Y, Z

Prepared from cleaned survey data · August 8, 2024

Executive Summary

This report delivers the Knowledge, Attitudes and Practices (KAP) survey results collected before and after intervention delivered to host-community households across three governorates (X, Y and Z). After cleaning, the dataset covers 1,439 household across 97 survey fields.
The comparison points to a substantial improvement across every indicator tracked. The most outcome in reported household diarrhea/cholera incidence over the prior two weeks, from 39.9% at baseline to 9.8% at endline — a reduction of about 75%. This improvement lines up with large gains in hygiene knowledge, handwashing practice, safe water treatment, and sanitation behavior, and with a shift in the primary drinking water source toward THE ORGANIZATION-rehabilitated infrastructure.
Key highlights:

- Diarrhea/cholera incidence fell from 39.9% to 9.8% of households (−30.1 points).
- Handwashing with soap and water rose from 22.9% to 94.5% of respondents.
- Correct identification of diarrhea causes and prevention methods rose 3–8x across items.
- 87.7% of Post-KAP households now rely on an THE ORGANIZATION-rehabilitated water network, versus 0% pre-intervention.
- Compliance with Sphere minimum water volume standards rose from 29.3% to 88.4% of households.
- 96.5% of Post-KAP respondents report getting enough water, and 73.8% feel access has "improved a lot."
- Gaps remain: 13% of Post-KAP respondents did not attend the hygiene promotion session, and demand remains high for additional infrastructure (sub-water networks, water tanks, bathrooms, sewage systems).

The sections below present the data by theme, followed by data quality notes and recommendations.

1. Introduction & Methodology

The data in this report is a household-level WASH KAP survey administered in two rounds: a Pre-KAP (baseline) round before an THE ORGANIZATION WASH intervention, and a Post-KAP (endline) round after the intervention. Each row in the dataset represents one household interview. The two rounds are not a matched panel (i.e., they are not guaranteed to be the same households interviewed twice), so results are interpreted as before/after comparisons at the population level rather than individual household change.

1.1 Data Preparation

Before analysis, the raw file (1,468 rows × 112 columns) was cleaned as follows:

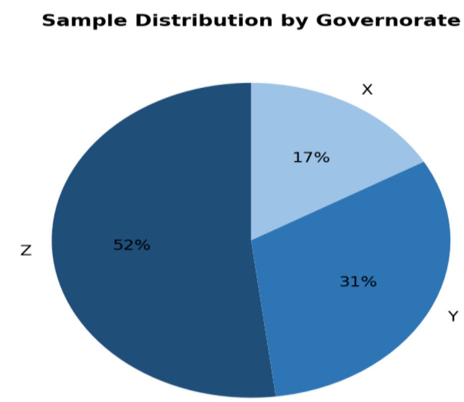
- 29 fully duplicate rows were removed.
- 15 columns that were 100% empty (group-header questions whose answers live in itemized Yes/No sub-columns) were removed.
- Inconsistent casing in the "Governorate Name" field (Y, Z, x) was standardized to X, Y, Z.
- 14 column headers with stray whitespace were trimmed.

The result dataset used for the "Family Size" category (small/medium/large) in the source data did not correspond reliably to the numeric household member count and was left as originally recorded; it is not used as an analytic variable in this report.

1.2 Reading This Report

Most indicators below are compared directly between Pre-KAP and Post-KAP respondents, since both rounds asked the same knowledge and practice questions. A smaller set of questions — satisfaction, perceived improvement, complaints-mechanism awareness, and remaining needs — were only asked in the Post-KAP round, since they concern the intervention itself; these are reported as Post-KAP-only figures and are marked accordingly.

2. Sample Demographics



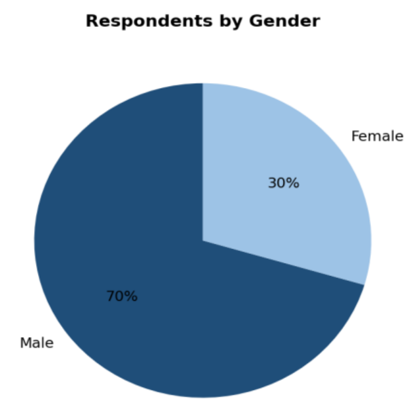


Figure 1–2. Sample composition by governorate and gender (combined Pre- and Post-KAP, n=1,439).

The household sample from three governorates, with Governorate Z contributing the largest share (52%), followed by Y (33%) and X (17%). Most respondents are male (72%), consistent with household heads typically responding to the survey. Average reported household size is similar across rounds (9.8 members pre-KAP vs. 10.1 post-KAP).

3. Water Access & Infrastructure

3.1 Main Drinking Water Source

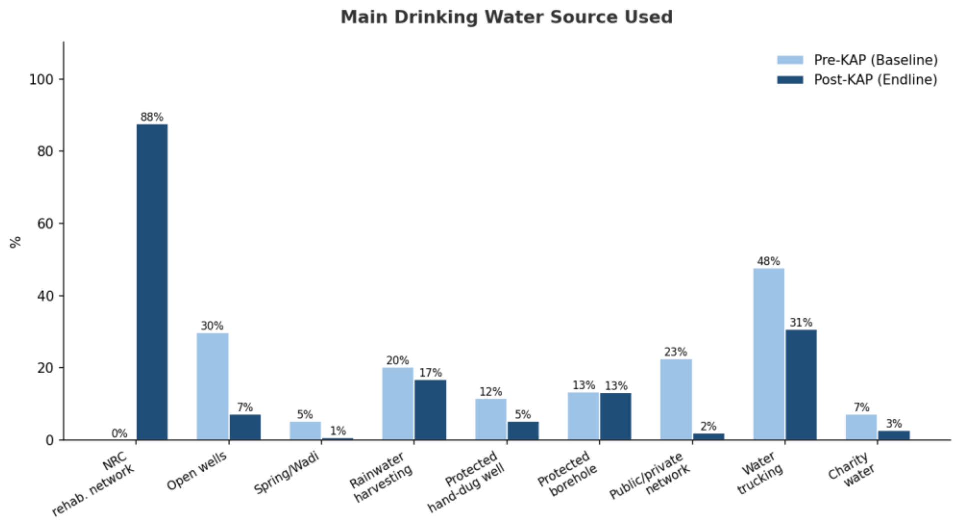


Figure 3. Share of households using each water source (households could report more than one source).

The most striking infrastructure shift is the emergence of THE ORGANIZATION-rehabilitated water pump networks as the dominant source post-intervention (87.7%, versus 0% pre-intervention, since this infrastructure did not yet exist). Reliance on informal or less safe sources fell accordingly: open wells (29.8% → 7.2%), water trucking (47.7% → 30.6%), and connection to public/private networks (22.6% → 2.0%, likely reflecting a shift of respondents onto the new THE ORGANIZATION network instead).

3.2 Distance and Volume vs. Sphere Standards

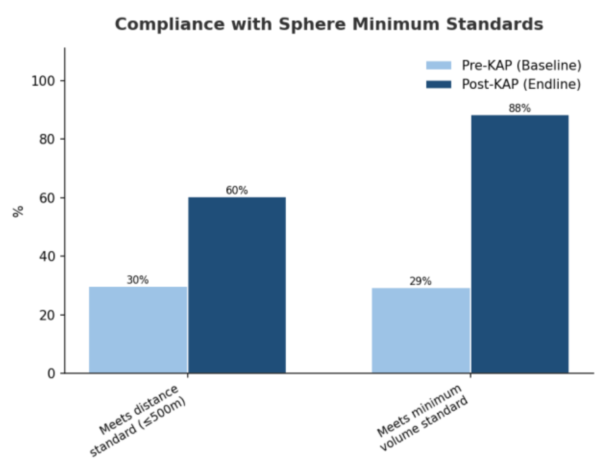


Figure 4. Share of households meeting the Sphere minimum standards for distance to water point (≤500m) and minimum daily water volume.

Compliance with the Sphere minimum distance standard rose from 29.8% to 60.3% of households, and compliance with the minimum volume standard rose sharply from 29.3% to 88.4%. Average actual volume collected per household rose from roughly 94 to 260 (reported units), while average distance to the nearest water point also improved.

3.3 Problems Accessing Sufficient Water

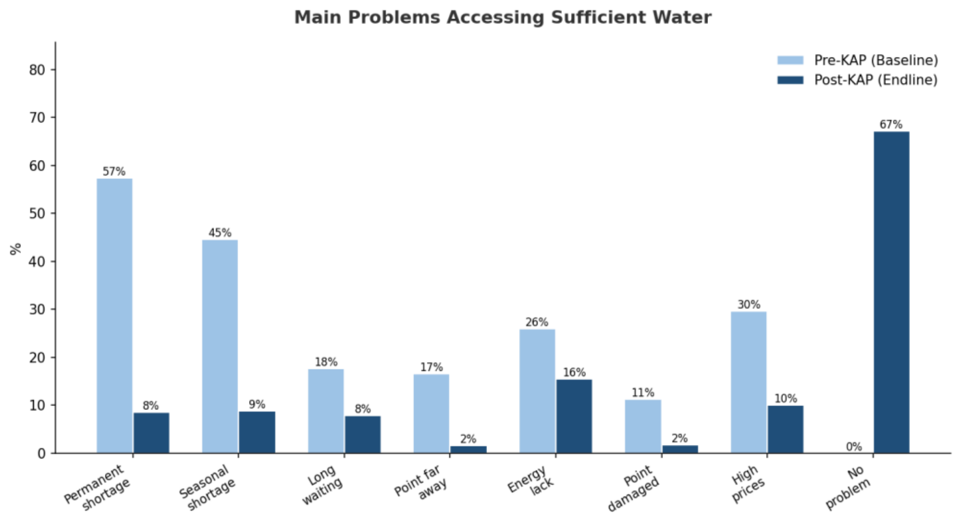


Figure 5. Main problems households report in accessing sufficient water (multiple responses allowed).

Every specific problem category declined post-intervention. Most notably, "permanent water shortage" fell from 57.4% to 8.5% of households, and "seasonal water shortage" fell from 44.6% to 8.8%. Correspondingly, the share of households reporting no problem at all accessing sufficient water rose from 0% pre-intervention to 67.2% post-intervention.

3.4 Perceptions of Water Access (Post-KAP only)

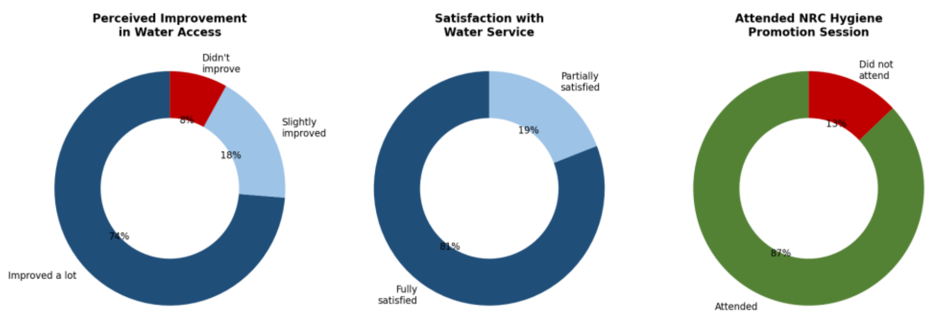


Figure 6. Post-KAP perceptions of water access improvement, satisfaction, and hygiene-session attendance.

- 96.5% of Post-KAP respondents say their household now gets enough water.
- 73.8% feel access "has improved a lot"; a further 18.2% feel it "slightly improved."
- 80.9% report being "fully satisfied" with the water service provided.
- 87.0% attended an THE ORGANIZATION hygiene promotion session; 13.0% did not.

4. Hygiene Knowledge & Practices

4.1 Recall of Key Hygiene Messages

Respondents were asked whether they could correctly recall specific hygiene messages. Post-KAP recall exceeded 80% on every item measured (cholera causes/prevention, personal & public hygiene, sewage management, garbage disposal, germ prevention, and general health messaging), versus recall in the 0–26% range pre-intervention for the same items (the lowest-recall items pre-intervention had not yet been introduced through the awareness sessions).

4.2 Handwashing Behavior

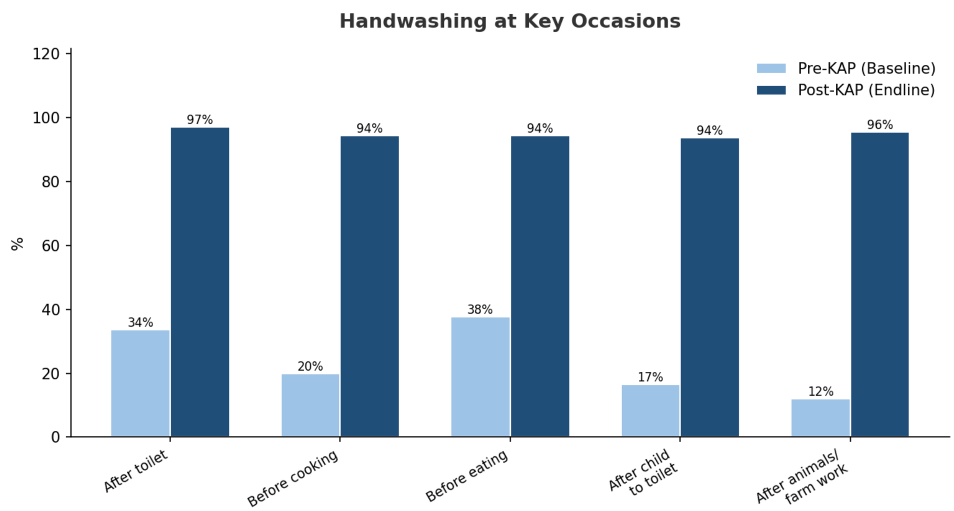


Figure 7. Share of households practicing handwashing at each key occasion.

Handwashing at all five critical occasions rose sharply. Handwashing after using the toilet rose from 33.7% to 97.2%, and — notably — the share of respondents who reported never washing their hands fell from 42.8% pre-intervention to 0% post-intervention.

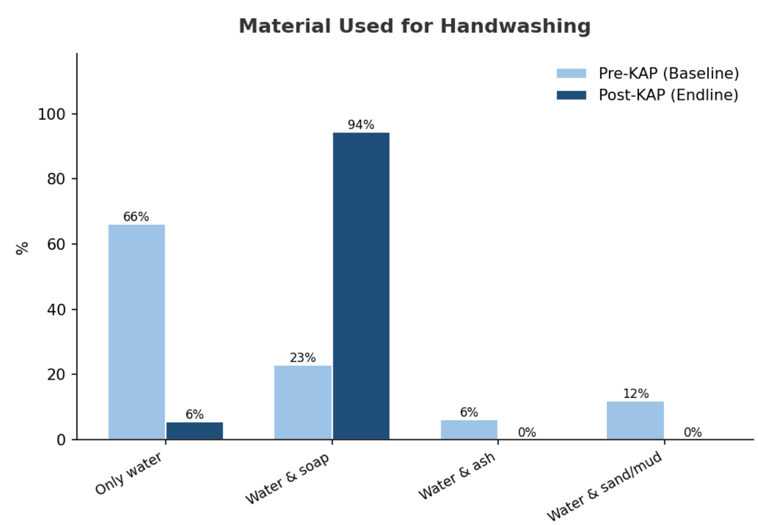


Figure 8. Material used for handwashing.

Use of water and soap together rose from 22.9% to 94.5% of households, while reliance on water alone (with no soap) fell from 66.2% to 5.5%. Among households not using soap, the most common pre-intervention reasons were that it was "too expensive" (33.1%), "not necessary" (33.4%), or "not available in market" (19.9%) — all of which fell to single digits or zero post-intervention.

5. Water Treatment & Safe Storage

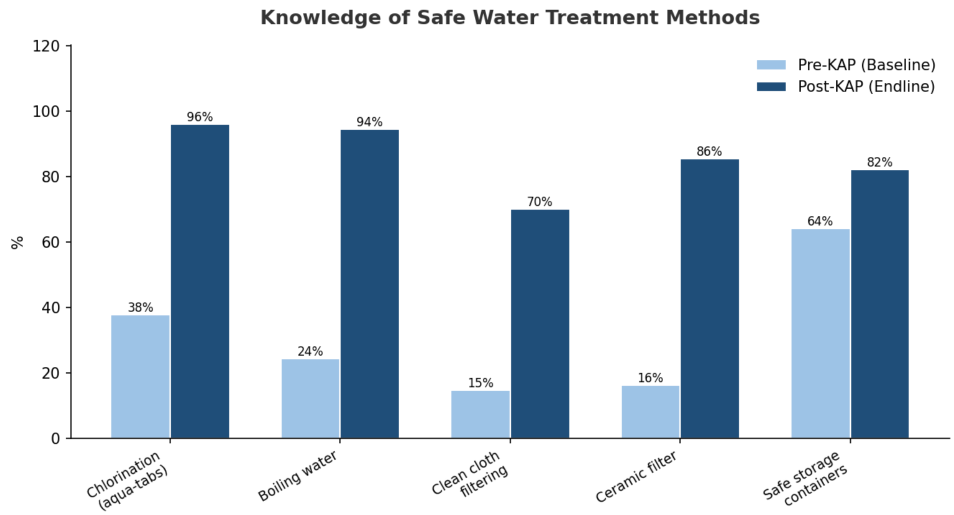


Figure 9. Knowledge of water treatment methods and use of safe storage containers.

Awareness of safe water treatment methods rose substantially: chlorination with aqua-tabs (37.6% → 95.9%), boiling (24.3% → 94.5%), filtering through clean cloth (14.6% → 69.9%), and ceramic filtering (16.2% → 85.5%). Reported use of clean containers to store water safely also rose, from 64.0% to 82.1% of households.

6. Sanitation: Garbage Disposal

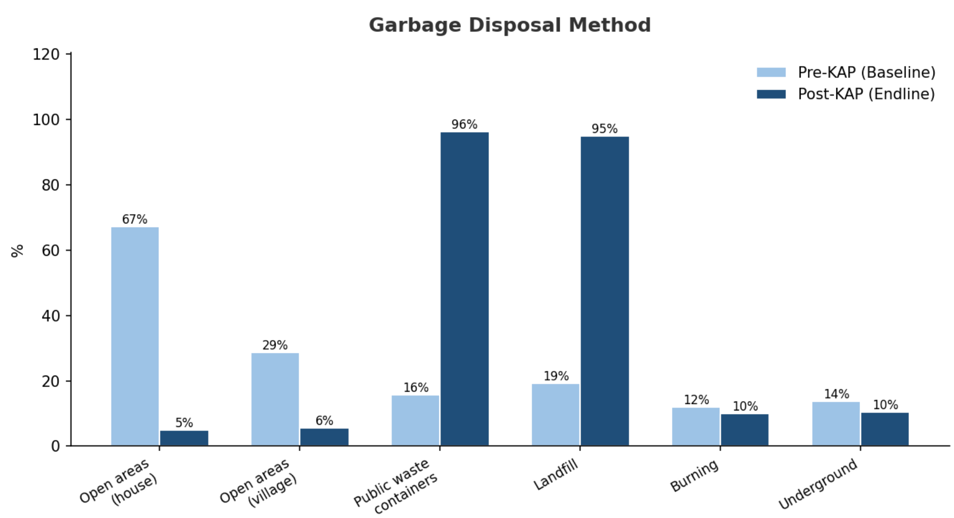


Figure 10. Method used to dispose of household garbage.

Disposal in open areas around the house fell from 67.2% to 5.0% of households, and disposal in open areas near the village fell from 28.7% to 5.7%. Use of proper disposal channels rose accordingly: public waste containers (15.7% → 96.2%) and landfill (19.3% → 94.9%). Correct knowledge of why proper disposal matters (disease prevention, fly control, and environmental/groundwater contamination) also rose from roughly 9–34% to roughly 88% across the three reasons.

7. Diarrhea: Knowledge, Prevention & Incidence

7.1 Household Incidence — Key Health Outcome

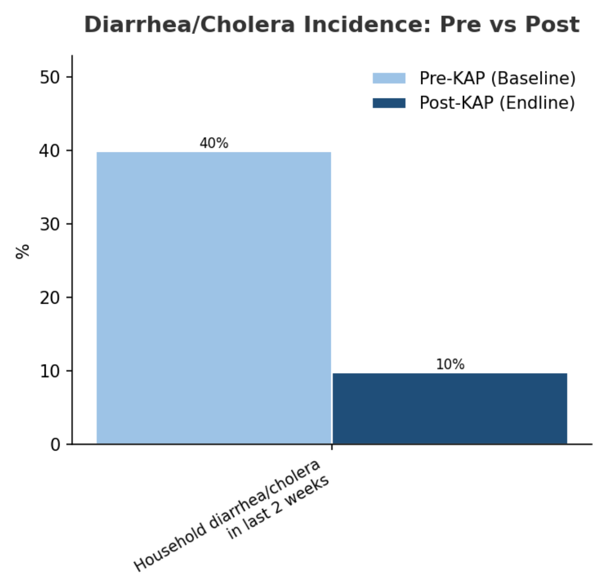


Figure 11. Share of households reporting a member with diarrhea/cholera (3+ loose stools/24h) in the prior two weeks.

This is the clearest downstream health outcome in the dataset. Reported diarrhea/cholera incidence in the previous two weeks fell from 39.9% of households pre-intervention to 9.8% post-intervention — a relative reduction of about 75%, consistent with the large gains in water access, water treatment, and hygiene practice documented above.

7.2 Knowledge of Causes

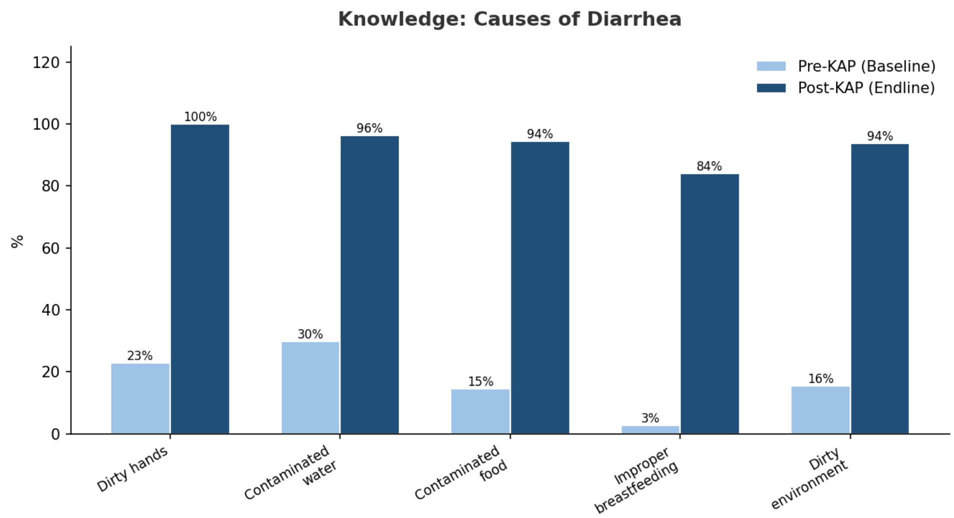


Figure 12. Share of respondents correctly identifying each cause of diarrhea.

Correct identification of diarrhea causes rose sharply across every item, most dramatically for "improper/partial breastfeeding" (2.6% → 84.0%) and "dirty surrounding environment" (15.5% → 93.8%). "Dirty/contaminated hands" reached 100% correct identification post-intervention.

7.3 Knowledge of Prevention

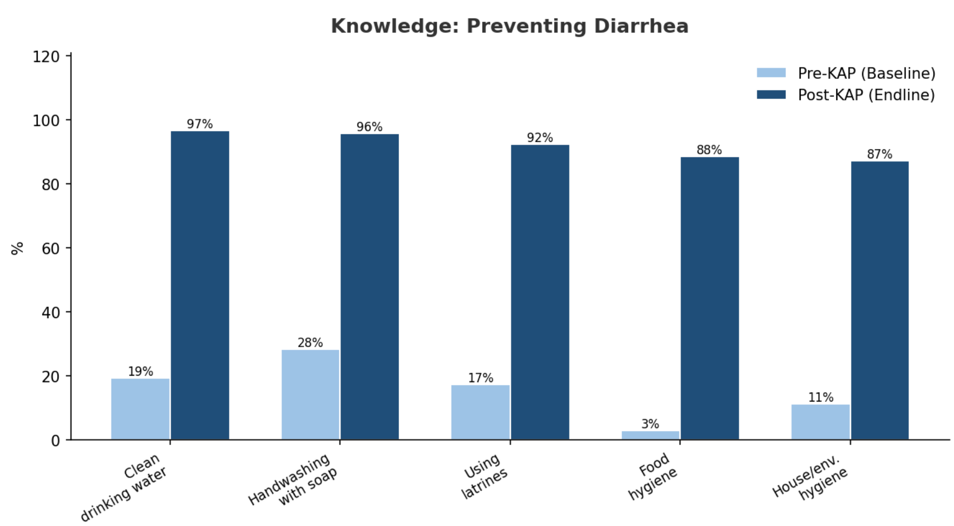


Figure 13. Share of respondents correctly identifying each diarrhea-prevention practice.

Prevention knowledge shows the same pattern, with the largest gains on "food hygiene" (2.9% → 88.5%) and "hygiene of house/surrounding environment" (11.3% → 87.2%).

7.4 Care-Seeking Behavior

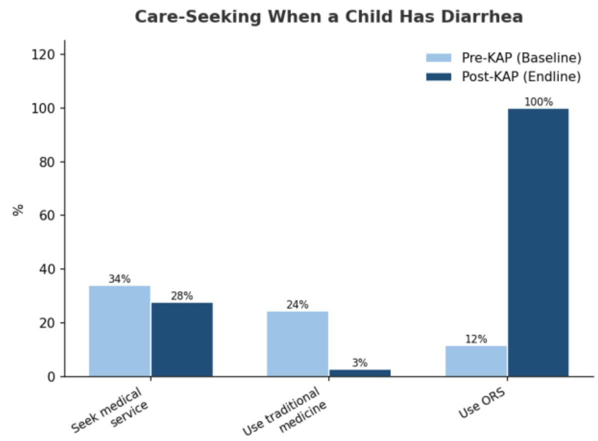


Figure 14. What households do when a child has diarrhea.

The most notable shift here is in ORS (oral rehydration solution) use, which rose from 11.6% to 100% of households reporting a diarrhea case — a strong indicator of improved case management. Reliance on traditional medicine fell from 24.5% to 2.8%. Reported medical-service-seeking declined slightly (34.1% → 27.8%), plausibly because fewer households are experiencing diarrhea at all post-intervention, and/or because ORS-based home management is now more prevalent for milder cases.

8. Community Engagement (Post-KAP only)

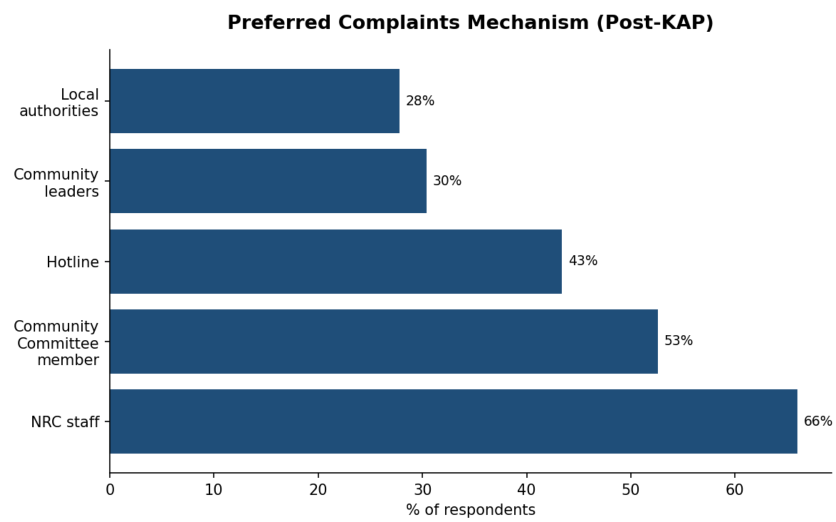


Figure 15. Preferred channel for raising complaints, among Post-KAP respondents.

95.9% of Post-KAP respondents report awareness of the complaints mechanism available through THE ORGANIZATION. The most preferred channels are direct contact with THE ORGANIZATION staff (66.0%) and the Community Committee (52.6%), followed by a dedicated hotline (43.4%), community leaders (30.4%), and local authorities (27.8%).

9. Remaining WASH Needs (Post-KAP only)

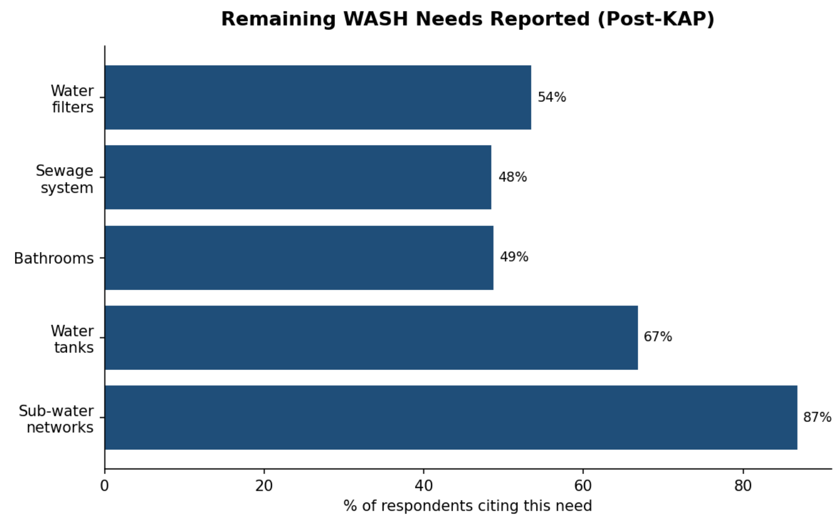


Figure 16. WASH services households report still needing, despite intervention gains.

Even with the improvements documented above, demand for further infrastructure remains high. The most commonly cited unmet need is installing/maintaining sub-water networks (86.8%), followed by household water tanks (66.8%), water filters (53.5%), bathrooms (48.7%), and sewage system installation/maintenance (48.5%). This suggests the intervention has meaningfully improved primary water access but has not yet closed gaps in complementary infrastructure.

10. Data Quality Notes

The following data-quality observations should be considered when interpreting results:

- The two survey rounds are independent samples, not a matched panel — comparisons reflect population-level change, not individual household change.
- 29 duplicate rows and 15 fully-empty columns were removed during cleaning (see the accompanying cleaned workbook and cleaning log for full detail).
- The "Family Size" category (small/medium/large) did not consistently correspond to the numeric household member count in the source data and was excluded from this analysis.
- 26 households reported 30+ members, which is plausible for extended/host-community households in this context but is noted here for awareness.
- All figures are self-reported by respondents and subject to recall and social-desirability bias, particularly for knowledge-recall questions.

11. Key Findings Summary

Indicator	Pre-KAP	Post-KAP	Change
Household diarrhea/cholera (last 2 weeks)	39.9%	9.8%	−30.1 pts
Handwashing with water & soap	22.9%	94.5%	+71.6 pts
Never wash hands	42.8%	0.0%	−42.8 pts
Meets Sphere volume standard	29.3%	88.4%	+59.1 pts

Indicator	Pre-KAP	Post-KAP	Change
Meets Sphere distance standard	29.8%	60.3%	+30.5 pts
Uses THE ORGANIZATION-rehabilitated network	0.0%	87.7%	+87.7 pts
Knows chlorination as treatment method	37.6%	95.9%	+58.3 pts
Proper garbage disposal (containers/landfill)	15.7–19.3%	94.9–96.2%	+~78 pts
Uses ORS for child diarrhea	11.6%	100.0%	+88.4 pts
No problem accessing sufficient water	0.0%	67.2%	+67.2 pts

12. Recommendations

- Sustain hygiene promotion outreach to close the remaining 13% attendance gap, prioritizing households that missed sessions.
- Prioritize sub-water network expansion and household water tanks, the two most-cited remaining needs, to extend gains beyond the primary network.
- Continue ORS distribution and promotion given its strong uptake (100% among households with a diarrhea case) and likely contribution to reduced severity.
- Investigate the modest decline in "seek medical service" (34.1% → 27.8%) to confirm it reflects appropriate home management rather than reduced access to care.
- Reconcile or redefine the "Family Size" categorical field with the survey team so it can be used reliably in future analysis.
- Consider a matched-panel design in future rounds (tracking the same households) to allow individual-level change to be measured directly, not just population-level shifts.